



Province of Alberta

# **PUBLIC HEALTH ACT**

Revised Statutes of Alberta 2000  
Chapter P-37

Current as of September 1, 2025

Office Consolidation

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## Note

All persons making use of this consolidation are reminded that it has no legislative sanction, that amendments have been embodied for convenience of reference only. The official Statutes and Regulations should be consulted for all purposes of interpreting and applying the law.

## Amendments Not in Force

This consolidation incorporates only those amendments in force on the consolidation date shown on the cover. It does not include the following amendments:

2020 c35 s30 amends ss1(1)(t) and 37(2)(a).

2025 c10 s47 amends ss1(1) and 5; repeals ss8 and 9; amends ss14(3) and 37(2)(a); repeals and substitutes s58.1(c)(v); repeals s62(8) and substitutes s62(8) and (8.1); amends ss63, 64(1) and (2) and 65(1) and (2); repeals s66.1(1)(b); amends ss66.2(1) and 74.

## Regulations

The following is a list of the regulations made under the *Public Health Act* that are filed as Alberta Regulations under the Regulations Act

|                                                                                                         | <b>Alta. Reg.</b> | <i>Amendments</i>                      |
|---------------------------------------------------------------------------------------------------------|-------------------|----------------------------------------|
| <b>Public Health Act</b>                                                                                |                   |                                        |
| Alberta Aids to Daily Living and<br>Extended Health Benefits.....                                       | 236/85 .....      | 109/87, 265/87,                        |
| NOTE: AR 154/2025 ss1 and 5<br>come into force on the coming into<br>force of s45(15) of the HSAA, 2025 |                   | 223/90, 109/2003,<br>22/2024, 154/2025 |
| Bodies of Deceased Persons.....                                                                         | 135/2008 .....    | 147/2017, 28/2021                      |

|                                                                                                                            |                |                                                                                                                                                                                                           |
|----------------------------------------------------------------------------------------------------------------------------|----------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Communicable Diseases .....                                                                                                | 238/85 .....   | 357/88, 37/98,<br>206/2001, 96/2005,<br>58/2006, 35/2007,<br>68/2008, 270/2009,<br>44/2014, 147/2016,<br>17/2018, 10/2019,<br>81/2019, 122/2021,<br>216/2022, 22/2024,<br>125/2024, 216/2024,<br>153/2025 |
| Emergency Powers.....                                                                                                      | 187/2009 ..... | 239/2018, 8/2024,<br>152/2025                                                                                                                                                                             |
| Food .....                                                                                                                 | 31/2006 .....  | 112/2006, 70/2015,<br>13/2018, 38/2018,<br>2/2020, 63/2020,<br>144/2021, 85/2022,<br>125/2024, 154/2025                                                                                                   |
| <i>NOTE: AR 154/2025 ss2 to 4 and<br/>6 to 10 come into force on the coming<br/>into force of s47(5) of the HSAA, 2025</i> |                |                                                                                                                                                                                                           |
| Housing.....                                                                                                               | 173/99 .....   | 251/2001, 354/2003,<br>158/2011, 140/2012,<br>155/2013, 161/2014,<br>127/2016, 38/2018,<br>144/2021, 154/2025                                                                                             |
| <i>NOTE: AR 154/2025 ss2 to 4 and<br/>6 to 10 come into force on the coming<br/>into force of s47(5) of the HSAA, 2025</i> |                |                                                                                                                                                                                                           |
| Immunization .....                                                                                                         | 182/2018 ..... | 153/2025                                                                                                                                                                                                  |
| Institutions.....                                                                                                          | 143/81 .....   | 317/82, 462/83,<br>109/2003, 216/2022,<br>98/2023                                                                                                                                                         |
| Nuisance and General Sanitation .....                                                                                      | 243/2003 ..... | 293/2006, 141/2012,<br>162/2014, 27/2019,<br>144/2021, 125/2024,<br>154/2025                                                                                                                              |
| <i>NOTE: AR 154/2025 ss2 to 4 and<br/>6 to 10 come into force on the coming<br/>into force of s47(5) of the HSAA, 2025</i> |                |                                                                                                                                                                                                           |
| Personal Services .....                                                                                                    | 1/2020 .....   | 36/2025, 154/2025                                                                                                                                                                                         |
| <i>NOTE: AR 154/2025 ss1 and 5<br/>come into force on the coming<br/>into force of s47(15) of the HSAA, 2025</i>           |                |                                                                                                                                                                                                           |
| Public Swimming Pools.....                                                                                                 | 204/2014 ..... | 27/2019, 46/2024,<br>154/2025                                                                                                                                                                             |
| <i>NOTE: AR 154/2025 ss2 to 4 and<br/>6 to 10 come into force on the coming<br/>into force of s47(5) of the HSAA, 2025</i> |                |                                                                                                                                                                                                           |
| Qualifications of Executive Officers .....                                                                                 | 51/99 .....    | 354/2003, 224/2011,<br>17/2018, 154/2025                                                                                                                                                                  |
| <i>NOTE: AR 154/2025 ss2 to 4 and<br/>6 to 10 come into force on the coming<br/>into force of s47(5) of the HSAA, 2025</i> |                |                                                                                                                                                                                                           |
| Recreation Area.....                                                                                                       | 198/2004 ..... | 85/2012, 127/2016                                                                                                                                                                                         |
| Treatment Services.....                                                                                                    | 248/85 .....   | 494/87, 17/99,<br>251/2001, 297/2003,<br>142/2010, 173/2012,<br>153/2014, 135/2017,<br>239/2018, 122/2021                                                                                                 |

|                                               |                |                                            |
|-----------------------------------------------|----------------|--------------------------------------------|
| Waiver.....                                   | 298/2003 ..... | 108/2004, 293/2006,<br>188/2009, 172/2012, |
| <i>NOTE: AR 154/2025 ss2 to 4 and</i>         |                | <i>152/2014, 204/2014,</i>                 |
| <i>6 to 10 come into force on the coming</i>  |                | <i>180/2019, 47/2024,</i>                  |
| <i>into force of s47(5) of the HSAA, 2025</i> |                | <i>154/2025</i>                            |
| Work Camps .....                              | 218/2002 ..... | 107/2011, 38/2018,                         |
| <i>NOTE: AR 154/2025 ss2 to 4 and</i>         |                | <i>144/2021, 154/2025</i>                  |
| <i>6 to 10 come into force on the coming</i>  |                |                                            |
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# **PUBLIC HEALTH ACT**

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### **Preamble**

WHEREAS the Government of Alberta is committed to providing leadership and support in the area of public health;

WHEREAS the Government of Alberta recognizes that taking measures to prevent, detect, assess and mitigate public health risks, including the spread of communicable diseases, epidemics and other illnesses, is essential to protect the health of Albertans;

WHEREAS the Government of Alberta recognizes the importance of a legal framework that provides for the prevention, detection, assessment and mitigation of public health risks;

WHEREAS the Government of Alberta recognizes the importance of effective and efficient provincial decision making, clearly defined roles and coordination among all parties involved in the management of public health during times of exigency;

WHEREAS the Government of Alberta is committed to transparency, accountability and responsiveness in the area of public health; and



WHEREAS the Government of Alberta is committed to protecting the health of Albertans, both in times of normalcy and in times of exigency, while still respecting the rights of Albertans under the *Alberta Bill of Rights*;

THEREFORE HER MAJESTY, by and with the advice and consent of the Legislative Assembly of Alberta, enacts as follows:

### Definitions

**1(1)** In this Act,

- (a) “biological agent” includes sera, immune globulins, vaccines and toxoids;
- (b) “Board” means the Public Health Appeal Board established under section 3;
- (c) “carrier” means a person who, without apparent symptoms of a communicable disease, harbours and may disseminate an infectious agent;
- (d) “certificate” means a compulsory examination and treatment certificate;
- (e) “Chief Medical Officer” means the Chief Medical Officer of Health appointed by the Minister under section 13;
- (e.1) repealed RSA 2000 cP-37 s77;
- (f) “communicable disease” means an illness in humans that is caused by an organism or micro-organism or its toxic products and is transmitted directly or indirectly from an infected person or animal or the environment;
- (g) “community health nurse” means a registered nurse within the meaning of the *Health Professions Act* who has the qualifications set out in the regulations and who is employed or engaged by a provincial health agency or a provincial health corporation under the *Provincial Health Agencies Act* or the Department to provide public health services;
- (h) repealed 2021 c15 s2;
- (i) “contamination” means the presence of an infectious agent on a body surface, or on or in an inanimate article or substance including food;

- (j) “council” means
  - (i) the council of a city, town, village, summer village, municipal district or Metis settlement,
  - (ii) in the case of an improvement district, the Minister responsible for the *Municipal Government Act* or a person that Minister designates in writing for the purpose, and
  - (iii) in the case of a special area, the Minister responsible for the *Special Areas Act* or a person that Minister designates in writing for the purpose;
- (k) “Department” means the Department of which the Minister is charged with the administration;
- (l) “Deputy Chief Medical Officer” means the Deputy Chief Medical Officer of Health appointed by the Minister under section 13;
- (m) “disinfection” means the destruction of infectious agents outside the body by any means;
- (n) “disinfestation” means the destruction or removal, by any physical or chemical process, of animal forms present on domestic animals or humans or in the environment;
- (o) “epidemic” means the occurrence in a community of persons of a number of cases of a communicable disease in excess of normal expectations;
- (p) “executive officer” means an executive officer within the meaning of section 9 or 16;
- (q) “facility” means any place where a person can receive treatment for communicable diseases;
- (r) “health practitioner” means any person who provides health care or treatment to any person;
- (r.1) “health services delivery organization” means a health services delivery organization under the *Provincial Health Agencies Act*;
- (s) repealed 2025 c10 s47(2);
- (t) “hospital” means an approved hospital as defined in the *Hospitals Act*;

- (u) “immunization” means the administration of a biological agent to a person to increase that person’s resistance to the effect of an infectious agent or its toxic products;
- (v) “infection” means the entry and multiplication of an infectious agent in the body of a person or animal;
- (w) “infectious agent” means an organism or micro-organism that is capable of producing a communicable disease;
- (x) “institution” means a correctional institution as defined in the *Corrections Act*, a facility as defined in the *Mental Health Act*, a continuing care home as defined in the *Continuing Care Act*, the premises where a child care program that is licensed under the *Early Learning and Child Care Act* is offered or provided and a hostel or other establishment operated to provide accommodation and maintenance for unemployed or indigent persons and, except in section 22, includes a supportive living accommodation as defined in the *Continuing Care Act*;
- (y) “isolation” means the separation of a person or animal infected with a communicable disease from other persons or animals in a place and under conditions that will prevent the direct or indirect conveyance of the infectious agent from the infected person or animal to a susceptible person or animal;
- (z) “laboratory” means a medical diagnostic laboratory where examinations of specimens of blood, spinal fluid, sputum, stool, urine, gastric washings, exudate or other specimen or discharge derived from a body are made for the purpose of determining the presence or absence of an infectious agent;
- (aa) “legal representative” means a lawyer, an executor or administrator of the estate of a deceased person, the trustee or guardian of a represented adult under the *Adult Guardianship and Trusteeship Act*, the agent designated in a personal directive made by a person in accordance with the *Personal Directives Act* and the trustee or guardian of a minor;
- (bb) “medical officer of health” means a physician appointed by the Minister under this Act as a medical officer of health, and includes the Chief Medical Officer and the Deputy Chief Medical Officer;

- (cc) “Minister” means the Minister determined under section 16 of the *Government Organization Act* as the Minister responsible for this Act;
- (dd) “municipality” means a city, town, village, summer village, municipal district, improvement district, special area and Metis settlement;
- (ee) “nuisance” means a condition that is or that might become injurious or dangerous to the public health, or that might hinder in any manner the prevention or suppression of disease;
- (ee.1) repealed 2005 c13 s8;
- (ff) “owner” means the registered owner, and any person in the actual or apparent possession or control of land or a premises;
- (ff.01) “peace officer” means a peace officer as defined in the *Peace Officer Act*;
- (ff.1) “police officer” means a police officer as defined in the *Police Act*;
- (gg) “prescribed”, with respect to a form, means a form that
  - (i) is in the form prescribed in the regulations, or
  - (ii) is in a form approved by the Minister, where no form has been prescribed in the regulations;
- (hh) “private place” means
  - (i) a private dwelling, and
  - (ii) privately owned land, whether or not it is used in connection with a private dwelling;
- (hh.01) “provincial health agency” means a provincial health agency under the *Provincial Health Agencies Act*;
- (hh.02) “provincial health corporation” means a provincial health corporation under the *Provincial Health Agencies Act*;
- (hh.1) “public health emergency” means an urgent and temporary occurrence or threat of an occurrence of
  - (i) an illness,

- (ii) a health condition,
  - (iii) an epidemic or pandemic disease,
  - (iv) a novel or highly infectious agent or biological toxin, or
  - (v) the presence of a chemical agent or radioactive material
- that poses a significant risk to the public of an increase in disease, injuries, disabilities or deaths in excess of expectations during times of normalcy;
- (ii) “public place” includes any place in which the public has an interest arising out of the need to safeguard the public health and includes, without limitation,
    - (i) public conveyances and stations and terminals used in connection with them,
    - (ii) places of business and places where business activity is carried on,
    - (iii) learning institutions,
    - (iv) institutions,
    - (v) places of entertainment or amusement,
    - (vi) places of assembly,
    - (vii) dining facilities and licensed premises,
    - (viii) accommodation facilities, including all rental accommodation,
    - (ix) recreation facilities,
    - (x) medical, health, personal and social care facilities, and
    - (xi) any other building, structure or place visited by or accessible to the public;
  - (jj) “quarantine” means
    - (i) in respect of persons or animals, the limitation of freedom of movement and contact with other persons or animals, and
    - (ii) in respect of premises, the prohibition against or the limitation on entering or leaving the premises,

during the incubation period of the communicable disease in respect of which the quarantine is imposed;

- (kk) “regional health authority” means a regional health authority under the *Provincial Health Agencies Act*;
- (ll) repealed 2002 c20 s2;
- (mm) “school” means
  - (i) a school operating under the *Education Act*,
  - (ii) a place where an early childhood services program is offered or provided, and
  - (iii) the premises where a child care program that is licensed under the *Early Learning and Child Care Act* is offered or provided;
- (nn) “sexually transmitted infections clinic” means a clinic operated by the Minister or a provincial health agency for the purposes of prevention and treatment of sexually transmitted infections;
- (oo) “teacher” includes an instructor, lecturer, professor, principal, president, supervisor or superintendent of any school, college, university, polytechnic institution, comprehensive community college or other learning institution;
- (pp) “terminal decontamination” means the decontamination of
  - (i) the clothing of a person,
  - (ii) the physical environment of a person,
  - (iii) the contents of the isolation room, and
  - (iv) any article or piece of equipment used in the diagnosis or treatment of a person

after the person has been removed from isolation or has ceased to be a source of infection or after isolation procedures have been discontinued.

**(2) Repealed RSA 2000 cP-37 s77.**

RSA 2000 cP-37 ss1,77;RSA 2000 cH-7 s147;2002 c20 s2; 2002 c32 s12;  
2005 c13 s8;2007 cC-10.5 s29;2008 cA-4.2 s149;2009 cS-23.5 s26;  
2012 cE-0.3 s282;2013 cS-19.3 s25;2016 c25 s2;2018 c19 s74;  
2020 c5 s2;2020 c13 s12;2020 c31 s40;2021 c15 s2;2022 cC-26.7 s77;  
2024 c10 s34;2024 c16 s20;2025 c10 s47(2)

**2** Repealed 2025 c10 s47(3).

## **Part 1**

### **Public Health Appeal Board**

#### **Board established**

**3(1)** There is hereby established a Public Health Appeal Board consisting of not more than 5 members who shall be appointed by the Lieutenant Governor in Council.

**(1.1)** A member must have the qualifications, if any, set out in the regulations.

**(2)** The Lieutenant Governor in Council shall designate the chair and vice-chair of the Board.

**(3)** The vice-chair shall act as chair in the event of the absence or inability to act of the chair.

**(4)** The members of the Board shall elect from their number an alternate vice-chair, who shall act as chair in the event of the absence or inability to act of the chair and vice-chair.

**(5)** The vice-chair and alternate vice-chair when acting under subsection (3) or (4) have the authority and duties of the chair.

**(6)** The members of the Board shall be appointed for terms not exceeding 3 years and may be reappointed for not more than 2 consecutive additional terms, each not exceeding 3 years.

**(7)** A member of the Board continues to hold office after the expiry of the member's term until

(a) the expiration of 30 days, or

(b) the member's successor is appointed,

whichever occurs first.

**(8)** Three members constitute a quorum at a meeting of the Board.

**(9)** The Board shall meet at the call of the chair or on a resolution of the Board.

**(10)** The Board may make rules governing the calling of meetings, the procedure to be used at meetings and the conduct of the meetings.

**(11)** Members of the Board

- (a) shall be paid remuneration at the rates prescribed by the Lieutenant Governor in Council, and
- (b) shall be paid their reasonable travelling and living expenses while absent from their ordinary places of residence and in the course of their duties as members of the Board, at the rates prescribed by the Lieutenant Governor in Council.

**(11.1)** Rates referred to in subsection (11) must be prescribed in accordance with any applicable regulations under the *Alberta Public Agencies Governance Act*.

**(11.2)** If regulations under the *Alberta Public Agencies Governance Act* establish rates in respect of remuneration or expenses referred to in subsection (11), those regulations prevail, to the extent of any conflict or inconsistency, over any regulations prescribing a rate under subsection (11).

**(12)** The Minister may provide clerical and secretarial services for the Board.

**(13)** Repealed 2003 c2 s1(26).

RSA 2000 cP-37 s3;2003 c2 s1(26);2009 cA-31.5 s68;2021 c15 s3

#### **Duties of Board**

**4(1)** The Board shall hear appeals pursuant to section 5.

**(2)** The Board may engage the services of persons having special technical, professional or other knowledge to assist it in the hearing of appeals.

1984 cP-27.1 s3;1998 c38 s5

#### **Appeal to Board**

**5(1)** In this section, “decision” means

- (a) an order issued under section 62, and
- (b) a decision to issue or to cancel, suspend or refuse to issue a licence, permit or other approval provided for in the regulations, and any other decision in respect of which an appeal to the Board is permitted under the regulations.

**(2)** A person who is directly affected by a decision may appeal the decision to the Board.

**(3)** A person referred to in subsection (2) shall commence the appeal by serving a notice of appeal in the prescribed form on the Board and the Minister within 10 days after receiving notice of the decision.



(3.1) Notice under subsection (3) is sufficiently served if it is left at an office of the Board or the Minister.

(4) Subject to subsections (5) and (6), the Board shall, if it is satisfied that the person making the appeal is directly affected by the decision, hear the appeal within 30 days after receiving the notice of appeal.

(5) Where the Board is satisfied that the appellant and the regional health authority, or either of them, have not made a reasonable effort to resolve the matters in dispute between them, it may refer the matter to the regional health authority for further consideration and redetermination.

(6) Where the Board refers a matter to the regional health authority under subsection (5), the Board may prescribe a time period within which the regional health authority must deal with the matter and may give to the regional health authority any other directions it considers appropriate.

(7) The Board shall provide the appellant, the regional health authority and, in a case where the decision or order appealed from was made by an employee or agent of the regional health authority, that employee or agent, an opportunity to appear and make representations orally or in writing, or both orally and in writing.

(8) The appellant, the regional health authority and, where the decision or order appealed from was made by an employee or agent of the regional health authority, that employee or agent, may be represented by counsel.

(9) Notwithstanding subsections (3) and (4), the Board may, if it considers it appropriate to do so, extend the time within which an appeal may be taken under subsection (3) or within which the Board must act under subsection (4).

(10) For the purposes of conducting an appeal under this section, the Board has all of the powers, privileges and immunities of a commissioner appointed under the *Public Inquiries Act*.

(11) The Board may confirm, reverse or vary the decision of the regional health authority and shall give written notice of its decision to the appellant and the regional health authority.

RSA 2000 cP-37 s5;2025 c10 s47(4)

#### **Stay pending appeal**

**6** An appeal taken pursuant to section 5 does not operate as a stay of the decision appealed from except so far as the chair or vice-chair of the Board so directs.

1984 cP-27.1 s5

**Annual report**

**7(1)** The Board shall make a report in each year to the Minister summarizing generally its activities and affairs in the preceding year.

**(2)** On receiving the report under subsection (1), the Minister shall lay a copy of it before the Legislative Assembly if it is then sitting, and if it is not then sitting, within 15 days after the commencement of the next sitting.

1984 cP-27.1 s6;1998 c38 s6

## **Part 2**

### **Administration**

**Alteration and disestablishment**

**8** The order disestablishing a health unit may contain any provisions the Lieutenant Governor in Council considers necessary

- (a) to provide for the transfer of the assets and property of the local board of the health unit to a regional health authority under the *Provincial Health Agencies Act*,
- (b) to provide for the assumption of liabilities and obligations of the local board of the health unit by a regional health authority under the *Provincial Health Agencies Act*, and
- (c) to facilitate the taking over of the affairs of the health unit by a regional health authority under the *Provincial Health Agencies Act*.

RSA 2000 cP-37 s8;2024 c10 s34

**Staff**

**9(1)** A regional health authority shall appoint one or more persons as medical officers of health and one or more persons as executive officers for the regional health authority for the purpose of carrying out this Act and the regulations.

**(2)** The Minister may appoint one or more persons as medical officers of health for a regional health authority if the Minister is of the opinion that the number of medical officers of health appointed by the regional health authority is insufficient.

**(3)** A person who is appointed as a medical officer of health under this section is, by virtue of the appointment, also an executive officer.

(4) A medical officer of health and an executive officer appointed under this section must have the qualifications, if any, set out in the regulations.

RSA 2000 cP-37 s9;2016 c25 s3;2021 c15 s4

#### **Provision of services by RHA**

**10** A provincial health agency or provincial health corporation shall provide the health promotional, preventive, diagnostic, treatment, rehabilitative and palliative services, supplies, equipment and care that the regulations require it to provide.

RSA 2000 cP-37 s10;2025 c10 s47(6)

**11** Repealed 2005 c13 s8.

#### **Provision of services by Minister**

**12** The Minister may provide to any person any health promotional, preventive, diagnostic, treatment, rehabilitative or palliative services, supplies, equipment and care and any drugs, medicines and biological agents prescribed in the regulations.

1984 cP-27.1 s22;1996 c31 s10

#### **Provision of services by individuals**

**12.1(1)** Subject to the regulations, an individual may provide a type of service set out in the regulations if the individual has the qualifications or meets the requirements set out in the regulations in respect of that type of service.

(2) Despite subsection (1), where the existence of a public health emergency has been confirmed under section 29(2.1) or where a state of public health emergency has been declared under section 52.1,

- (a) the Minister or Chief Medical Officer may by order specify qualifications that an individual must have or requirements that an individual must meet in order to provide a type of service instead of or in addition to any qualifications or requirements set out in the regulations, and
- (b) an individual may provide the type of service referred to in clause (a) if
  - (i) the individual has the qualifications or meets the requirements specified in the order made under clause (a), and

- (ii) the individual has the qualifications or meets the requirements set out in the regulations that the order made under clause (a) requires the individual to have or meet, if any, in order to provide the service.

2021 c15 s5

**Chief Medical Officer**

**13(1)** The Minister may appoint a person as Chief Medical Officer of Health and a person as Deputy Chief Medical Officer of Health for the purposes of this Act.

**(1.1)** The Chief Medical Officer must

- (a) be a physician,
- (b) have
  - (i) a certificate, diploma or degree in public health, or
  - (ii) training and practical experience that in the Minister's opinion is equivalent to a certificate, diploma or degree in public health,
- (c) be a fellow of the Royal College of Physicians and Surgeons of Canada, and
- (d) have any additional qualifications set out in the regulations.

**(1.2)** The Deputy Chief Medical Officer must have the qualifications, if any, set out in the regulations.

**(2)** The Deputy Chief Medical Officer may act in the place of the Chief Medical Officer during the Chief Medical Officer's temporary absence or temporary inability to act.

**(3)** The Chief Medical Officer may in writing delegate to the Deputy Chief Medical Officer any power, duty or function conferred or imposed on the Chief Medical Officer under this Act or the regulations.

RSA 2000 cP-37 s13;2021 c15 s6

**Powers of Chief Medical Officer**

**14(1)** The Chief Medical Officer

- (a) shall, on behalf of the Minister, monitor the health of Albertans and make recommendations to the Minister, provincial health agencies, provincial health corporations and health services delivery organizations on measures to protect and promote the health of the public and to prevent disease and injury,

- (b) shall act as a liaison between the Government and provincial health agencies, provincial health corporations, health services delivery organizations, medical officers of health and executive officers in the administration of this Act,
- (c) shall monitor activities of provincial health agencies, provincial health corporations, health services delivery organizations, medical officers of health and executive officers in the administration of this Act, and
- (d) may give directions to provincial health agencies, provincial health corporations, health services delivery organizations, medical officers of health and executive officers in the exercise of their powers and the carrying out of their responsibilities under this Act.

(2) Where the Chief Medical Officer is of the opinion that a medical officer of health or executive officer is not properly exercising powers or carrying out duties under this Act in respect of a matter, the Chief Medical Officer may assume the powers and duties of the medical officer of health or executive officer in respect of the matter and act in that person's place.

(3) Where the Chief Medical Officer decides to act under subsection (2), the Chief Medical Officer shall forthwith give a notice in writing setting out the reasons why the Chief Medical Officer has so decided to

- (a) the medical officer of health or executive officer,
- (b) where applicable, the regional health authority by whom the medical officer or executive officer is employed or for whom the medical officer of health or executive officer acts as agent, and
- (c) the Minister.

(4) Notwithstanding anything in this section, the Chief Medical Officer has all the power and authority conferred on a medical officer of health or an executive officer under this Act and the regulations and may exercise that power and authority for any purpose under this Act or the regulations.

RSA 2000 cP-37 s14;2009 c13 s2;2025 c10 s47(7)

#### **Diseases under surveillance**

##### **15(1) Where**

- (a) a disease is not prescribed as a notifiable disease under the regulations, and

- (b) the Chief Medical Officer considers that it is advisable to keep the disease under surveillance in order to assess the impact of the disease and the need for further intervention under this Act,

the Chief Medical Officer may by notice in writing require a medical officer of health, a physician or a director of a laboratory to provide to the Chief Medical Officer or to a medical officer of health, or to both, at the times and in the manner set out in the notice any information in respect of the disease that is set out in the notice.

- (2) A person who receives a notice under subsection (1) shall comply with it.

RSA 2000 cP-37 s15;2016 c25 s4

#### **Order making Act applicable**

**15.1(1)** Notwithstanding anything in this Act, the Minister may, on the advice of the Chief Medical Officer, by order, make any provision of this Act or the regulations applicable in respect of a particular disease if the Minister is satisfied that the disease presents a serious threat to public health.

(1.1) The Minister shall post online an order made under subsection (1) as soon as is reasonably possible after the order is made.

- (2) The *Regulations Act* does not apply in respect of an order referred to in subsection (1).

2002 c32 s12;2021 c15 s7

#### **Administrative powers**

**16(1)** The Minister may appoint one or more physicians as medical officers of health.

(2) A person who is appointed as a medical officer of health under subsection (1) is, by virtue of the appointment, also an executive officer.

(3) The Minister may designate one or more persons employed in the Department as executive officers.

(4) A medical officer of health appointed under this section and an executive officer designated under this section must have the qualifications, if any, set out in the regulations.

RSA 2000 cP-37 s16;2016 c25 s5;2021 c15 s8;2025 c10 s47(8)

#### **Inspection by Minister**

**17** The Minister and employees of the Government authorized by the Minister for the purpose may

- (a) make inquiries into the management and affairs of a provincial health agency, provincial health corporation or health services delivery organization,
- (b) enter and inspect any place under the jurisdiction of a provincial health agency, provincial health corporation or health services delivery organization, and
- (c) examine the records of a provincial health agency, provincial health corporation or health services delivery organization

for the purpose of verifying the accuracy of reports and ensuring that this Act and the regulations are complied with.

RSA 2000 cP-37 s17;2025 c10 s47(9)

#### **Provision of information**

**18(1)** Where a medical officer of health reasonably believes that a person has engaged in or is engaging in any activity that is causing or may cause a threat to the health of the public or a class of the public, the medical officer of health may by notice in writing require the person to provide to the medical officer of health within the time specified in the notice any information respecting the activity that is specified in the notice.

**(2)** A person who receives a notice under subsection (1) shall comply with it.

1998 c38 s10

#### **Disclosure of information**

**18.1(1)** In this section,

- (a) “charter school”, “early childhood services program”, “independent student”, “independent school” and “board” have the meanings given to them in the *Education Act*;
- (b) “child care program” means a child care program in respect of which a licence has been issued under the *Early Learning and Child Care Act*.

**(2)** A medical officer of health may by notice in writing require a board, a person responsible for the operation of an independent school or of a charter school, an operator of an early childhood services program or a provider of a child care program to provide to the medical officer of health, in the form and manner and within the time specified in the notice, the information set out in subsection (2.1) that is in its custody or within its control, for the purpose of contacting a parent or guardian of a student or child, or contacting an independent student, regarding voluntary health

programs, including immunization, hearing, vision, speech and dental health programs, and for the purpose of communicable diseases control.

**(2.1)** For the purposes of subsection (2), a medical officer of health may require the following information to be provided:

- (a) the name, address, postal code, date of birth and sex, and the grade level, if applicable, of a student or child and the school, early childhood services program or child care program, as the case may be, attended by the student or child;
- (b) the name, address, postal code, telephone number and electronic address
  - (i) of the parent or guardian of a child or a student other than an independent student, or
  - (ii) of an independent student;
- (c) any other information prescribed in the regulations.

**(3)** A board, a person responsible for the operation of an independent school or charter school or an operator of an early childhood services program or a provider of a child care program who receives a notice under subsection (2) shall comply with it.

2009 c13 s3;2012 cE-0.3 s282;2016 c25 s6;2020 c31 s40;  
2025 c6 s41

#### **Provision of information by Minister of Education**

**18.2(1)** The Minister may require the Minister of Education to provide to the Minister, in the form and manner and within the time specified, the information set out in subsection (2) that is in the custody or under the control of the Department of Education, for the purpose of contacting a parent or guardian of a student, or contacting an independent student, respecting voluntary health programs, including immunization, hearing, vision, speech and dental health programs, and for the purpose of communicable diseases control.

**(2)** For the purposes of subsection (1), the Minister may require the following information to be provided:

- (a) a student's name, address, postal code, date of birth, sex, grade level and school;
- (b) the name, address, postal code, telephone number and electronic address



- (i) of the parent or guardian of a student other than an independent student, or
- (ii) of an independent student;
- (c) any other information prescribed in the regulations.

2016 c25 s7

### **Part 3**

## **Communicable Diseases and Public Health Emergencies**

#### **Reporting re immunization**

**18.3** A health practitioner who performs an immunization or conducts an assessment in respect of immunization shall report information respecting immunization to the Minister in accordance with the regulations.

2016 c25 s8

#### **Reporting adverse event following immunization**

**18.4** A health practitioner shall, in accordance with the regulations, report any adverse event following immunization of which the health practitioner becomes aware in respect of a person to whom the health practitioner has provided or is providing professional services.

2016 c25 s8

#### **Immunization schedules**

**18.5** A health practitioner who performs an immunization shall comply with

- (a) the most recent version of an immunization schedule published by the Chief Medical Officer, or
- (b) the part of the most recent version of an immunization schedule published by the Chief Medical Officer

that is identified by the Chief Medical Officer as being mandatory.

2016 c25 s8

#### **Biological agent storage, handling, transportation**

**18.6(1)** A person who stores, handles or transports, or directs the storage, handling or transportation of, a biological agent intended for use in immunization shall do so in accordance with the regulations.

**(2)** A person referred to in subsection (1) who becomes aware of a contravention of the regulations referred to in subsection (1) shall take action in accordance with the regulations.

2016 c25 s8

**Information to medical officer of health**

**19(1)** Where a medical officer of health knows or has reason to believe

- (a) that a person suffering from a communicable disease is or may be in or has frequented or may have frequented a public place, or
- (b) that a public place may be contaminated with a communicable disease,

the medical officer of health may by notice in writing to the person in charge of the public place require that person to provide to the medical officer of health within the time specified in the notice any information relating to the public place, the person and the communicable disease that is specified in the notice.

**(2)** A person who receives a notice referred to in subsection (1) shall comply with it.

1998 c38 s12

**Information respecting public health emergency**

**19.1(1)** Where a medical officer of health

- (a) knows of or has reason to suspect the existence of, or the threat of the existence of, a public health emergency, and
- (b) has reason to believe that a person has information relevant to the public health emergency that will assist the medical officer of health in carrying out duties and exercising powers under section 29 in respect of the public health emergency,

the medical officer of health or an executive officer or community health nurse designated for that purpose by the medical officer of health may, by notice in writing, require the person who has the information to provide the information that is specified in the notice to the medical officer of health, executive officer or community health nurse.

**(2)** A person who receives a notice referred to in subsection (1) shall comply with it.

2002 c32 s12

**Discovery and treatment of infection**

**20(1)** Every person who knows or has reason to believe that the person is or may be infected with a communicable disease prescribed in the regulations for the purposes of this subsection shall immediately consult a physician to determine whether the

person is infected or not, and if the person is found to be infected, shall submit to the treatment directed and comply with any other conditions prescribed by the physician until the physician is satisfied that the person is not infectious.

(2) Every person who knows or has reason to believe that the person is or may be infected with a sexually transmitted infection prescribed in the regulations for the purposes of this subsection shall immediately consult a physician or attend a sexually transmitted infections clinic to determine whether the person is infected or not, and if the person is found to be infected, shall submit to the treatment directed and comply with any other conditions prescribed by a physician until the physician is satisfied that the person is not infectious.

(3) A person is subject to the duties imposed under subsections (1) and (2) with respect to minor children under the person's custody, care or control.

RSA 2000 cP-37 s20;2016 c25 s9

#### **Notification of change of address**

**21** During a period in which a person or a minor under the person's custody, care or control is required by section 20 to submit to treatment or to comply with conditions, that person shall immediately notify the consulting physician, the clinic or a medical officer of health of any change in the person's address or the address of the minor, as the case may be.

RSA 2000 cP-37 s21;2016 c25 s10;2025 c10 s47(10)

#### **Notification of communicable disease**

**22(1)** Where a health practitioner, a teacher or a person in charge of an institution knows or has reason to believe that a person under the care, custody, supervision or control of the health practitioner, teacher or person in charge of an institution is infected with a communicable disease prescribed in the regulations for the purposes of this subsection, the health practitioner, teacher or person in charge of an institution shall notify a medical officer of health

- (a) by the fastest means possible in the case of a prescribed disease that is designated in the regulations as requiring immediate notification, or
- (b) within 48 hours in the prescribed form in the case of any other prescribed disease.

**(1.1)** Where an operator of a supportive living accommodation as defined in the *Continuing Care Act* knows or has reason to believe that a person residing at the supportive living accommodation is

infected with a communicable disease prescribed in the regulations for the purposes of subsection (1), the operator shall notify a medical officer of health in accordance with subsection (1)(a) and (b).

(2) Where a physician, a nurse practitioner or a midwife knows or has reason to believe that a person under the care in a hospital of the physician, nurse practitioner or midwife is infected with a disease to which subsection (1) applies, the physician, nurse practitioner or midwife shall, in addition to carrying out the physician's, nurse practitioner's or midwife's responsibilities under subsection (1), immediately inform the medical director or other person in charge of the hospital, and the medical director shall notify a medical officer of health by telephone or in accordance with the prescribed form.

(3) Where a physician, a community health nurse, a nurse practitioner, a midwife or a person in charge of an institution knows or has reason to believe that a person under the care, custody, supervision or control of the physician, community health nurse, nurse practitioner, midwife or person in charge of an institution is infected with a disease referred to in section 20(2), the physician, community health nurse, nurse practitioner, midwife or person in charge of an institution shall, within 48 hours, notify a medical officer of health in the prescribed form.

(4) Where an operator of a supportive living accommodation as defined in the *Continuing Care Act* knows or has reason to believe that a person residing at the supportive living accommodation is infected with a disease referred to in section 20(2), the operator shall, within 48 hours, notify a medical officer of health in the prescribed form.

RSA 2000 cP-37 s22;2002 c20 s4;2007 c23 s4;  
2009 cS-23.5 s26;2016 c25 s11;2022 cC-26.7 s77

#### **Discovery in laboratory**

**23** Where an examination of a specimen derived from a human body reveals evidence of a communicable disease, the director of the laboratory conducting the examination shall,

- (a) in the case of a disease prescribed in the regulations for the purposes of this clause, notify a medical officer of health
  - (i) by the fastest means possible in the case of a prescribed disease that is designated in the regulations as requiring immediate notification, or
  - (ii) within 48 hours in the prescribed form or by telephone, in the case of any other prescribed disease,

and

- (b) in the case of a disease referred to in section 20(2), notify a medical officer of health in the prescribed form within 48 hours.

RSA 2000 cP-37 s23;2016 c25 s12

#### **Submission of specimens to Provincial Laboratory**

**24** Where examination of a specimen at a laboratory indicates the existence or possible existence of a communicable disease prescribed in the regulations for the purposes of this section, the director of the laboratory conducting the examination shall ensure that a sample, together with a description of the type of examination that was carried out, is provided to the Provincial Laboratory of Public Health in accordance with the regulations.

1998 c38 s13

#### **Disease notification**

**25** If a medical officer of health receives notification of a suspected case of a communicable disease referred to in section 20(1) or sexually transmitted infection referred to in section 20(2) located outside the area of Alberta over which the medical officer of health has jurisdiction, that medical officer of health shall immediately notify a medical officer of health having jurisdiction over the area of Alberta where the suspected case is located.

RSA 2000 cP-37 s25;2016 c25 s13;2025 c10 s47(11)

#### **Notification of epidemics and other threats**

**26** A health practitioner, a teacher or a person in charge of an institution who knows of or has reason to suspect the existence of

- (a) a communicable disease in epidemic form,
- (b) another illness or health condition occurring at an unusually high rate, or
- (c) a communicable disease or another illness or health condition that is caused by a nuisance or other threat to the public health

shall immediately notify a medical officer of health by the fastest means possible.

RSA 2000 cP-37 s26;2002 c32 s12;2007 c23 s4;2016 c25 s14;  
2025 c10 s47(12)

#### **Duty to notify Chief Medical Officer**

**27(1)** Where a medical officer of health receives

- (a) notification under section 26(a), or

- (b) notification of a communicable disease that is designated in the regulations as requiring immediate notification

the medical officer of health shall immediately notify the Chief Medical Officer by the fastest means possible.

**(2)** Where a medical officer of health receives a notification under section 26(b) or (c) and reasonably believes that the illness, communicable disease or health condition constitutes a significant risk to the public health, the medical officer of health shall immediately notify the Chief Medical Officer by the fastest means possible.

RSA 2000 cP-37 s27;2002 c32 s12

**28** Repealed 2025 c10 s47(13).

### **Isolation, Quarantine and Special Measures**

#### **Isolation and quarantine**

**29(1)** A medical officer of health who knows of or has reason to suspect the existence of a communicable disease or a public health emergency in an area of Alberta over which the medical officer of health has jurisdiction may initiate an investigation to determine whether any action is necessary to protect the public health.

**(2)** Where the investigation confirms the presence of a communicable disease, the medical officer of health

- (a) shall carry out the measures that the medical officer of health is required by this Act and the regulations to carry out, and
- (b) may do any or all of the following:
  - (i) take whatever steps the medical officer of health considers necessary
    - (A) to suppress the disease in those who may already have been infected with it,
    - (B) to protect those who have not already been exposed to the disease,
    - (C) to break the chain of transmission and prevent spread of the disease, and
    - (D) to remove the source of infection;

- (ii) where the medical officer of health determines that a person or class of persons engaging in the following activities could transmit an infectious agent, prohibit the person or class of persons from engaging in the activity by order, for any period and subject to any conditions that the medical officer of health considers appropriate:
  - (A) attending a school;
  - (B) engaging in the occupation of the person or the class of persons, subject to subsection (2.01);
  - (C) having contact with any persons or any class of persons;
- (iii) issue written orders for the decontamination or destruction of any bedding, clothing or other articles that have been contaminated or that the medical officer of health reasonably suspects have been contaminated.

**(2.01)** An order made under subsection (2)(b)(ii)(B) does not prevent a person who is subject to the order from engaging in the person's occupation if the person is able to do so without attending any location, having any contact or engaging in any activity that could transmit an infectious agent.

**(2.1)** Where the investigation confirms the existence of a public health emergency, the medical officer of health

- (a) has all the same powers and duties in respect of the public health emergency as he or she has under subsection (2) in the case of a communicable disease, and
- (b) may take whatever other steps are, in the medical officer of health's opinion, necessary in order to lessen the impact of the public health emergency.

**(2.11)** Where a state of public health emergency has been declared under section 52.1 and the Lieutenant Governor in Council has acted under subsection (2.12),

- (a) any orders issued by a medical officer of health under subsection (2.1) may only be respecting a specific person or persons or specific public place related to the nature of the public health emergency, and
- (b) a medical officer of health must cease acting under subsection (2.1) with respect to classes of persons in respect of a declared state of public health emergency under section 52.1.

**(2.12)** Notwithstanding subsection (2.1), where a state of public health emergency has been declared under section 52.1, the Lieutenant Governor in Council, taking into account any advice of the Chief Medical Officer, may, in respect of all persons or a class of persons, including a class of individuals, bodies corporate, associations, non-profit or for-profit organizations,

- (a) take whatever steps are necessary in order to lessen the impact of the public health emergency,
- (b) do anything referred to in subsection (2)(b), and
- (c) by order reverse or vary any order issued by a medical officer of health under subsection (2.1) related to the public health emergency whether that order was issued before or during the declared state of public health emergency under section 52.1.

**(2.13)** The Lieutenant Governor in Council may in writing exempt a person or class of persons from the application of an order made under subsection (2.12).

**(2.14)** Where an order under subsection (2.12) or an exemption under subsection (2.13) is not made in respect of a specific person or persons, the Lieutenant Governor in Council shall provide a copy of the order or exemption to the Minister as soon as is reasonably possible.

**(2.15)** An order made under subsection (2.12) or an exemption made under subsection (2.13) may incorporate, adopt or declare in force a code, standard, guideline, schedule or body of rules as amended or replaced from time to time, including a code, standard, guideline, schedule or body of rules developed by the Minister or the Chief Medical Officer, that relates to the order or exemption.

**(2.2)** Subject to subsection (2.21), a medical officer of health or the Chief Medical Officer may in writing exempt a person or class of persons from the application of an order made under subsection (2) or (2.1) by that medical officer of health.

**(2.21)** During a declared state of public health emergency under section 52.1, a medical officer of health or the Chief Medical Officer may exempt only a specific person or persons from an order of the medical officer of health or the Chief Medical Officer made under subsection (2.1).

**(2.3)** Where an order under subsection (2) or (2.1) or an exemption under subsection (2.2) is not made in respect of a specific person or persons, the medical officer of health who makes the order or



exemption, or the Chief Medical Officer if the Chief Medical Officer makes the exemption, shall provide a copy of the order or exemption to the Minister as soon as is reasonably possible.

**(3)** A medical officer of health shall forthwith notify the Chief Medical Officer of any action taken under subsection (2)(b) or of the existence of a public health emergency.

**(3.1)** On being notified of the existence of a public health emergency under subsection (3) the Chief Medical Officer shall forthwith notify the Minister.

**(4)** The jurisdiction of a medical officer of health extends to any person or animal, whether or not the person resides or the animal is located in an area of Alberta over which the medical officer of health has jurisdiction,

(a) who is known or suspected by a medical officer of health

(i) to be infected with a communicable disease, illness or health condition,

(ii) to be a carrier,

(iii) to have been in contact with an infected person or animal or a contaminated environment to a sufficient degree to have had the opportunity to become infected with a communicable disease, illness or health condition,

(iv) to be susceptible to and at risk of contact with a communicable disease, illness or health condition, or

(v) to be exposed to a chemical agent or radioactive material,

or

(b) who has been determined by the Chief Medical Officer to be at risk of infection with a communicable disease, illness or health condition.

**(5)** An order made under subsection (2) or (2.1) or an exemption made under subsection (2.2) may incorporate, adopt or declare in force a code, standard, guideline, schedule or body of rules as amended or replaced from time to time, including a code, standard, guideline, schedule or body of rules developed by the Minister or the Chief Medical Officer, that relates to the order or exemption.

**(6)** The *Regulations Act* does not apply to an order made under subsection (2), (2.1), (2.11) or (2.12) or an exemption made under

subsection (2.13), (2.2) or (2.21) or to a code, standard, guideline, schedule or body of rules that the order or exemption incorporates, adopts or declares in force.

**(7)** If an order under subsection (2), (2.1), (2.11) or (2.12) or an exemption under subsection (2.13), (2.2) or (2.21) is not made in respect of a specific person or persons, the Minister shall

- (a) post the order or exemption online as soon as is reasonably possible after the order or exemption is made, and
- (b) ensure that any code, standard, guideline, schedule or body of rules that is incorporated, adopted or declared in force by the order or exemption is readily available to the public.

RSA 2000 cP-37 s29;2002 c32 s12;2021 c15 s9;2023 c11 s2;  
2025 c10 s47(14)

#### **Validation of orders**

**29.1(1)** An order made under section 29(2)(b)(i) or (2.1) before the coming into force of this section is validated and declared for all purposes to have been validly made as of the date on which the order was made.

**(2)** Everything done before the coming into force of this section under or in reliance on an order made under section 29(2)(b)(i) or (2.1) is validated and declared for all purposes to have been validly done.

**(3)** Any code, standard, guideline, schedule or body of rules incorporated, adopted or declared in force by an order made under section 29(2)(b)(i) or (2.1) before the coming into force of this section is validated and declared for all purposes to have been validly incorporated, adopted or declared in force as of the date on which the order was made.

2021 c15 s10

#### **Entry for examination**

**30(1)** Where a medical officer of health knows or has reason to believe that

- (a) a person suffering from a communicable disease referred to in section 20 may be found in any place, or
- (b) that any place may be contaminated with such a communicable disease,

the medical officer of health may enter that place without a warrant for the purpose of conducting an examination to determine the existence of the communicable disease.

**(2)** Where a medical officer of health is conducting an examination pursuant to subsection (1), the medical officer of health may

- (a) order the detention of any person, and
- (b) order the closure of the place, including any business that is carried on in it,

until the medical officer of health has completed the investigation, but not for a period of more than 24 hours.

**(3)** When the medical officer of health is not able to complete the investigation within 24 hours, the medical officer of health may make an application to a judge of the Court of Justice for an order to extend the period of detention or closure under subsection (2) for an additional period of not more than 7 days, and the judge may make the order accordingly.

RSA 2000 cP-37 s30;2008 c32 s26;AR 75/2023

#### **Examination**

**31(1)** Where a medical officer of health knows or has reason to believe that a person may be infected with a communicable disease referred to in section 20, that person shall, at the request of the medical officer of health, submit to any examinations necessary to determine whether the person is infected with the disease.

**(2)** In conducting an examination pursuant to subsection (1) to determine the existence of a communicable disease, the medical officer of health may require from any person who has knowledge of it the production of any information concerning the disease, including the sources or suspected sources of the disease and the names and addresses of any persons who may have been exposed to or become infected with the disease.

1984 cP-27.1 s41

#### **Notices**

**32(1)** A medical officer of health may cause to be placed warning notices in the prescribed form in, on, at or near any place in which a person is isolated or quarantined, or which requires decontamination or destruction.

**(2)** No person shall remove a warning notice placed in accordance with this section unless the person has the consent of a medical officer of health.

1984 cP-27.1 s42

#### **Effect of isolation or quarantine**

**33(1)** Where a person infected with a communicable disease requires isolation or quarantine as prescribed in the regulations, the

person shall be isolated or quarantined in a hospital or other place approved for the purpose by a medical officer of health.

**(2)** No person who is suffering from a communicable disease for which isolation or quarantine is required under the regulations shall remain or be permitted to remain in any public place, other than a hospital or other place approved under subsection (1), unless a medical officer of health is satisfied that the presence of the person in the public place would involve no risk to the public health.

**(2.1)** and **(2.2)** Repealed RSA 2000 cP-37 s77.

**(3)** Where a person is isolated or quarantined in

- (a) a social care facility,
- (a.1) a supportive living accommodation as defined in the *Continuing Care Act*,
- (b) a food handling establishment, or
- (c) living accommodation attached to a social care facility, supportive living accommodation or food handling establishment,

a medical officer of health may, by notice to the owner of the social care facility, supportive living accommodation or food handling establishment, order the owner not to operate or permit the operation of the social care facility, supportive living accommodation or food handling establishment until decontamination of the social care facility, supportive living accommodation or food handling establishment is completed.

**(4)** Where a person is isolated or quarantined in a place under circumstances that require terminal decontamination, a medical officer of health may, by notice to the owner of the place, order the owner to refuse entry to the place to any person other than

- (a) an executive officer,
- (b) a medical officer of health, or
- (c) a person with the consent of an executive officer or a medical officer of health

until decontamination is completed.

RSA 2000 cP-37 ss33,77;2009 cS-23.5 s26;2016 c25 s15;  
2020 c13 s12;2020 c27 s10;2022 cC-26.7 s77

**Provision of services**

**34(1)** When a person is isolated or quarantined in a hospital or other place approved for the purpose by a medical officer of health, the medical officer of health shall ensure that the person is provided with all supplies and services necessary for the person's health and subsistence.

**(2)** The medical officer of health shall ensure that any person providing supplies or services pursuant to this section takes adequate precaution to avoid contracting the communicable disease.

**(3)** Where the Minister is satisfied that it would cause undue hardship to require a person to whom supplies or services are provided under this section to pay for them, the Minister may pay for all or part of the cost of the supplies or services.

RSA 2000 cP-37 s34;2016 c25 s16

**Decontamination**

**35** No person shall

- (a) remove anything from a place in respect of which decontamination is required, or
- (b) give, lend, sell or offer for sale anything that has been exposed to contamination

until decontamination has been completed to the satisfaction of a medical officer of health.

RSA 2000 cP-37 s35;2016 c25 s17

**Transportation of infected person**

**36** A person transporting another person who that person knows or has reason to believe is suffering from a communicable disease requiring isolation or quarantine under the regulations shall inform a medical officer of health having jurisdiction over the area of Alberta in which the person is being transported and comply with any conditions respecting the transportation that are prescribed by the medical officer of health.

RSA 2000 cP-37 s36;2016 c25 s18;2025 c10 s47(15)

**Epidemics****Notification of epidemic**

**37(1)** When a medical officer of health is of the opinion that

- (a) a communicable disease is in epidemic form, and
- (b) hospital facilities within the area are inadequate to provide the necessary isolation or quarantine facilities,

the medical officer of health shall immediately inform the Minister.

(2) On the recommendation of the Minister, the Lieutenant Governor in Council

- (a) may order a board of an approved hospital as defined in the *Hospitals Act* to provide isolation or quarantine accommodation in the amount and manner prescribed in the order, and
- (b) may order the owner of a facility to provide isolation or quarantine accommodation in the amount and manner prescribed in the order.

(3) Where an order is made pursuant to subsection (2)(b), any reasonable expense incurred by the owner of a facility in compliance with the order is the responsibility of the Crown in right of Alberta.

1984 cP-27.1 s47;1996 c31 s20

#### Order of Lieutenant Governor in Council

**38(1)** Where the Lieutenant Governor in Council is satisfied that a communicable disease referred to in section 20(1) has become or may become epidemic or that a public health emergency exists, the Lieutenant Governor in Council may do any or all of the following:

- (a) order the closure of any public place;
- (b) subject to the *Legislative Assembly Act* and the *Senatorial Selection Act*, order the postponement of any intended election for a period not exceeding 3 months.
- (c) repealed 2021 c15 s11.

(2) Where an election is postponed under subsection (1), the order shall name a date for holding the nominations or polling, or both of them, and nothing in the order adversely affects or invalidates anything done or the status of any person during the period of time between the date of the order and the completion of the election.

(3) Repealed 2021 c15 s11.

RSA 2000 cP-37 s38;2002 c32 s12;2021 c15 s11

#### Recalcitrant Patients

##### Issue of certificate

**39(1)** Where a physician, community health nurse, midwife or nurse practitioner knows or has reason to believe that a person

- (a) is infected with a disease prescribed in the regulations for the purposes of this section, and

- (b) refuses or neglects
  - (i) to submit
    - (A) to a medical examination for the purpose of ascertaining whether the person is infected with that disease, or
    - (B) to medical, surgical or other remedial treatment that has been prescribed by a physician and that is necessary to render the person non-infectious, or
  - (ii) to comply with any other conditions that have been prescribed by a physician as being necessary to mitigate the disease or limit its spread to others,

the physician, community health nurse, midwife or nurse practitioner shall immediately notify a medical officer of health having jurisdiction over the area of Alberta in which the person is located in the prescribed form.

**(2)** The medical officer of health shall issue a certificate in the prescribed form where the medical officer of health is satisfied, taking into account any evidence that the medical officer of health considers relevant, that

- (a) the person meets the criteria set out in subsection (1)(a) and (b),
- (b) the person poses a risk to mitigating the disease or limiting its spread to others, and
- (c) there is no other reasonable means of mitigating the disease or limiting its spread to others.

**(3)** A certificate pursuant to subsection (2) must be issued within 72 hours of the date of service of the notification pursuant to subsection (1).

**(4)** Where the physician referred to in subsection (1) is a medical officer of health having jurisdiction over the area of Alberta in which the allegedly infected person is located, the physician may issue the certificate referred to in subsection (2).

**(5)** A person in respect of whom a certificate is issued may apply to a judge of the Court of King's Bench at any time for cancellation of the certificate.

**(6)** The application shall be served on

- (a) the medical officer of health who issued the certificate, and
- (b) the chief executive officer of the facility in which the applicant is detained, if the applicant is under detention at the time of the application,

not less than 2 days before the application is returnable.

(7) Notwithstanding subsection (6), a judge of the Court, on the ex parte application of the person referred to in subsection (5), may dispense with the service of the application under subsection (6) or authorize the giving of a shorter period of notice.

(8) Where the judge considers it appropriate to do so, the judge may order that the application under subsection (5) be heard in private.

(9) The judge may grant or refuse the order applied for and may make any other order the judge considers appropriate.

RSA 2000 cP-37 s39;2002 c20 s5;2009 c53 s149;2016 c25 s19;  
2021 c15 s12;AR 217/2022;2025 c10 s47(17)

#### **Authority of certificate**

**40(1)** A certificate is authority

- (a) for any peace officer to apprehend the person named in it and convey the person to any facility specified by the medical officer of health who issued the certificate within 7 days from the date the certificate is issued,
- (b) for a physician to perform any test or physical examination required to determine whether that person has a communicable disease and to detain that person at the facility for the period required to obtain the result of the examination,
- (c) for any physician to treat or prescribe treatment for that person in order to render that person non-infectious, with or without the consent of the person, and to detain the person for that purpose, and
- (d) for a physician to prescribe any other conditions necessary to mitigate the disease or limit its spread to others.

(2) The medical director of, or in the medical director's absence the attending physician at, a facility to which a person is conveyed under subsection (1) shall ensure that the person is examined under that section within 24 hours after the person's arrival at the facility.



**(3)** Where a person is detained pursuant to a certificate, the medical director of the facility in which the person is detained shall forthwith

- (a) inform the person or the person's guardian, if any, of
  - (i) the reason for the issuance of the certificate, and
  - (ii) the name and address of the facility in which the person is detained,
- (b) advise the person or the person's guardian, if any, that the person has a right to retain and instruct counsel without delay, and
- (c) give the person or the person's guardian, if any, a copy of section 39.

RSA 2000 cP-37 s40;2007 c23 s4;2016 c25 s20;2021 c15 s13

#### **Release**

**41(1)** Subject to subsection (2), a person who is detained in a facility pursuant to a certificate shall be released not later than 7 days after the date the person is admitted to the facility pursuant to the certificate, unless an isolation order is issued under section 44.

**(2)** A person who is detained in a facility pursuant to a certificate shall be released forthwith if the physician who examines the person certifies

- (a) that there is no evidence of active disease, or
- (b) that, although there is evidence of active disease, the physician is satisfied that the person will comply with the treatment and any other conditions ordered by the physician in a manner that will ensure the protection of the public health.

1984 cP-27.1 s51

#### **Notification of release**

**42** Where a person is released pursuant to section 41, the physician who examined the patient or the medical director of the facility shall, on the release of the patient, forthwith notify the medical officer of health who issued the certificate of the circumstances of the release.

1984 cP-27.1 s52

#### **Treatment after release**

**43(1)** Where a person is released pursuant to section 41(2)(b), the person shall comply with the treatment and any other conditions

that are prescribed by any physician assigned by the medical director of the facility.

**(2)** Where a person who has been required to submit to treatment or comply with conditions following the person's release fails to undergo treatment or comply with the conditions, a medical officer of health may issue an order in the prescribed form to a peace officer or other person to apprehend that person and return that person to the facility.

**(3)** On receipt of an order under subsection (2), a peace officer or other person is empowered to arrest without warrant the person named in it and return that person to the facility.

**(4)** Sections 41 and 42 and subsections (1), (2) and (3) apply to a person who is arrested and returned to a facility under subsection (3).

1984 cP-27.1 s53;1988 c41 s15

#### **Isolation order**

**44(1)** Subject to subsection (1.1), where one physician supported by a laboratory report demonstrating evidence of an infectious agent certifies or 2 physicians certify that a person is infected with an organism that produces a disease prescribed in the regulations for the purposes of this section and that the person refuses or neglects

- (a) to submit to medical, surgical or other remedial treatment, or
- (b) to comply with any other conditions

that have been prescribed by a physician as being necessary to mitigate that disease or to limit its spread to others, the physician or physicians shall each issue an isolation order in the prescribed form.

**(1.1)** A physician or physicians shall not issue an isolation order under subsection (1) unless the physician is or the physicians are satisfied that there is no other reasonable means of mitigating the disease or limiting its spread to others.

**(2)** Subsection (1) applies whether or not there is a certificate in existence in respect of the person who is the subject of the isolation order or orders.

**(3)** A physician issuing an isolation order shall forthwith send a copy of the isolation order to the Chief Medical Officer.

1984 cP-27.1 s54;1988 c41 s16;1998 c38 s19;2021 c15 s14

**Authority of isolation order**

**45(1)** An isolation order under section 44 is authority for a health practitioner to observe, examine, care for, treat, obtain biological specimens from, control and detain in a facility the person named in it with or without that person's consent until the order is cancelled under section 46.

**(2)** A person in respect of whom isolation is ordered under section 44 shall be re-examined by a physician at least once every 7 days to ascertain whether the person may be released under section 46.

RSA 2000 cP-37 s45;2007 c23 s4

**Cancellation of isolation order**

**46(1)** Where, after separate examinations by each of them, 2 physicians are of the opinion that a person in respect of whom isolation has been ordered under section 44

- (a) is not infectious, or
- (b) will comply with the conditions of the person's discharge,

the 2 physicians shall issue an order in the prescribed form cancelling the isolation order.

**(2)** Immediately on issuing an order cancelling an isolation order, the physicians who signed the order shall send a copy of it to the Chief Medical Officer.

1984 cP-27.1 s56;1988 c41 s18;1998 c38 s19

**Warrant for examination**

**47(1)** Any person who has reasonable and probable grounds to believe that a person

- (a) is infected with a disease prescribed in the regulations for the purpose of this section, and
- (b) refuses or neglects
  - (i) to submit
    - (A) to a medical examination for the purpose of ascertaining whether the person is infected with the disease, or
    - (B) to medical, surgical or other remedial treatment that has been prescribed by a physician and that is necessary to render the person non-infectious, or

- (ii) to comply with any other conditions that have been prescribed by a physician as being necessary to mitigate the disease or limit its spread to others,

may bring an information under oath before a judge of the Court of Justice.

**(2)** Where an information is brought before a judge of the Court of Justice under subsection (1) and the judge is satisfied that the person with respect to whom the information is brought should be examined in the interests of the person's own health or the health of others and that the examination cannot reasonably be arranged in any other way, the judge may issue a warrant in the prescribed form to apprehend that person for the purpose of the examination.

**(3)** A warrant under this section may be directed to any peace officer and shall name or otherwise describe the person with respect to whom the warrant is issued.

**(4)** Where a peace officer apprehends a person pursuant to a warrant under this section, the person is deemed to be a person in respect of whom a certificate has been issued under section 39.

RSA 2000 cP-37 s47;2008 c32 s26;AR 75/2023

#### **Duty on issue of isolation order**

**48** Where a person is detained pursuant to an isolation order or orders, the medical director of the facility in which the person is detained shall forthwith

- (a) inform the person or the person's guardian, if any, of
  - (i) the reason for the issuance of the isolation order or orders, and
  - (ii) the name and address of the facility in which the person is detained,
- (b) advise the person or the person's guardian, if any, that the person has a right to retain and instruct counsel without delay, and
- (c) give the person or the person's guardian, if any, a copy of section 49.

RSA 2000 cP-37 s48;2021 c15 s15

#### **Application to Court for cancellation**

**49(1)** A person in respect of whom isolation is ordered may apply to a judge of the Court of King's Bench at any time for cancellation of the isolation order or orders.

(2) The application shall be served on

- (a) the physician or physicians who issued the isolation order or orders, and
- (b) the chief executive officer of the facility in which the applicant is a patient

not less than 7 days before the application is returnable.

(3) Notwithstanding subsection (2), a judge of the Court, on the ex parte application of the person referred to in subsection (1), may dispense with the service of the application under subsection (2) or authorize the giving of a shorter period of notice.

(4) Where the judge considers it appropriate to do so, the judge may order that the application under subsection (2) be heard in private.

(5) The judge may grant or refuse the order applied for and may make any other order the judge considers appropriate.

RSA 2000 cP-37 s49;2009 c53 s149;AR 217/2022

#### **Unauthorized absence**

**50(1)** Where a person in respect of whom isolation has been ordered leaves the facility and leave of absence has not been granted by the medical director of the facility, the medical director may issue an order in the prescribed form to a peace officer or other person ordering the return of the person to the facility.

(2) An order issued pursuant to subsection (1) is sufficient authority for the person to whom it is directed to apprehend the person named in it and return the person to the facility.

(3) A person who is returned to a facility under this section may be detained until the conditions under section 46 have been met.

1984 cP-27.1 s60

#### **Transfer to another facility**

**51(1)** The medical director of the facility in which a person is detained may, for reasons of treatment or in compliance with the person's wishes, transfer the person to another facility, on completing a memorandum of transfer in the prescribed form.

(2) Where a person is transferred under subsection (1), the authority to detain, control and treat the person continues in force in the facility to which the person is transferred.

1984 cP-27.1 s61

**Leave of absence**

**52(1)** The medical director or an attending physician at a facility in which a person is detained may grant the person a leave of absence from the facility subject to any terms and conditions prescribed by the medical director or attending physician to ensure that the public health is protected.

**(2)** Where a person is on a leave of absence granted under this section and it appears to the medical director or the attending physician that the person is not complying with the conditions to which the leave of absence is subject, the medical director or attending physician may revoke the leave of absence and recall the person to the facility.

**(3)** Section 50 applies in the case of a person who has been recalled under subsection (2) and fails to return to the facility in accordance with the instructions of the medical director or attending physician.

1984 cP-27.1 s62

**State of Public Health Emergency****State of public health emergency**

**52.1** Where, on the advice of the Chief Medical Officer, the Lieutenant Governor in Council is satisfied that

- (a) a public health emergency exists or may exist, and
- (b) prompt co-ordination of action or special regulation of persons or property is required in order to protect the public health,

the Lieutenant Governor in Council may make an order declaring a state of public health emergency relating to all or any part of Alberta.

2002 c32 s12;2007 c23 s4;2013 c10 s37;2020 c5 s3;2021 c15 s16

**Local state of public health emergency**

**52.2** If, on the advice of a medical officer of health and in consultation with the Chief Medical Officer, the Minister is of the opinion that

- (a) a public health emergency exists or may exist in an area of Alberta, and
- (b) prompt coordination of action or special regulation of persons or property is required in order to protect the public health,

the Minister may make an order declaring a local state of public health emergency relating to all or part of that area of Alberta.  
2002 c32 s12;2016 c25 s21;2025 c10 s47(18)

**52.21** Repealed 2021 c15 s17.

**Contents of order**

**52.3** An order under section 52.1 or 52.2 must identify the nature of the public health emergency and the area to which it relates.  
2002 c32 s12

**Publication of order**

**52.4** The Minister shall publish and make available the details of an order under section 52.1 or 52.2 in the manner the Minister considers appropriate.  
2002 c32 s12;2007 c23 s4;2021 c15 s18;2025 c10 s47(19)

**52.5** Repealed 2025 c10 s47(20).

**Powers during emergency**

**52.6(1)** On the making of an order under section 52.1 and for up to 60 days following the lapsing of that order, the Minister, a provincial health agency or a provincial health corporation may do any or all of the following for the purpose of preventing, combating or alleviating the effects of the public health emergency and protecting the public health:

- (a) acquire or use any real or personal property;
- (b) authorize or require any qualified person to render aid of a type the person is qualified to provide;
- (c) repealed 2021 c15 s19;
- (d) authorize the entry into any building or on any land, without warrant, by any person;
- (e) provide for the distribution of essential health and medical supplies and provide, maintain and co-ordinate the delivery of health services.

**(1.01)** On the making of an order under section 52.2 and during the state of public health emergency, the Minister, provincial health agency or provincial health corporation may exercise any or all of the powers set out in subsection (1)(a), (b), (d) or (e) for the purpose of preventing, combating or alleviating the effects of the public health emergency and protecting the public health.

**(1.1)** On the making of an order under section 52.1 in respect of pandemic disease and for up to 60 days following the lapsing of that order, the Chief Medical Officer may, subject to any terms and conditions the Chief Medical Officer may impose, authorize the absence from employment of any persons

- (a) who are ill with pandemic disease,
- (b) who are caring for a family member ill with pandemic disease, or
- (c) repealed RSA 2000 cP-37 s77.

**(1.2)** Repealed RSA 2000 cP-37 s77.

**(2)** Nothing in this section limits or abrogates the operation of any other provision in this Act or the regulations that imposes a duty or confers a power on any person.

RSA 2000 cP-37 s77;2002 c32 s12;2007 c23 s4;2020 c13 s12;2021 c15 s19;  
2025 c10 s47(21)

**52.61** Repealed RSA 2000 cP-37 s77.

#### **Compensation**

**52.7(1)** Where the Minister, a provincial health agency or provincial health corporation acquires or uses real or personal property under section 52.6 or where real or personal property is damaged or destroyed due to the exercise of any powers under that section, the Minister, provincial health agency or provincial health corporation shall pay reasonable compensation in respect of the acquisition, use, damage or destruction.

**(2)** If any dispute arises concerning the amount of compensation payable under subsection (1) the matter is to be determined by arbitration, and the *Arbitration Act* applies in such a case.

2002 c32 s12;2025 c10 s47(22)

#### **Termination of public health emergency order**

**52.8(1)** An order under section 52.1 lapses, unless continued by a resolution of the Legislative Assembly, at the earlier of the following:

- (a) at the end of 30 days, but if the order is in respect of pandemic disease, at the end of 90 days;
- (b) when the order is terminated by the Lieutenant Governor in Council.



(2) Where, on the advice of the Chief Medical Officer, the Lieutenant Governor in Council considers that a public health emergency no longer exists in an area in relation to which an order was made under section 52.1, the Lieutenant Governor in Council shall make an order terminating the declaration in respect of that area.

2002 c32 s12;2007 c23 s4;2021 c15 s21

#### **Termination of state of local public health emergency**

**52.81(1)** The Minister may terminate an order made under section 52.2 at any time the Minister considers appropriate in the circumstances.

(2) An order under section 52.2 ceases to be of any force or effect on the making of an order under section 52.1 relating to the same area of Alberta.

(3) An order under section 52.2 lapses at the end of 30 days unless

- (a) it is terminated earlier by the Minister, or
- (b) it is renewed for an additional period not exceeding 30 days.

(4) Section 52.4 applies to the renewal of an order under section 52.2.

(5) If, on the advice of a medical officer of health and in consultation with the Chief Medical Officer, the Minister is of the opinion that a public health emergency no longer exists in an area for which an order under section 52.2 was made, the Minister shall make an order terminating the declaration in respect of that area.

2002 c32 s12;2016 c25 s21;2025 c10 s47(23)

**52.811** Repealed 2021 c15 s22.

#### **Publication**

**52.82** Immediately after an order is made under section 52.8(2) or 52.81(5), the Minister shall cause the details of the order to be published by any means of communication that the Minister considers will make the details of the order known to the majority of the population of the area affected by the termination order.

2002 c32 s12;2025 c10 s47(24)

#### **Regulations Act**

**52.83** The *Regulations Act* does not apply to an order made under section 52.1 or 52.2.

2002 c32 s12;2007 c23 s4;2021 c15 s23

**Regulations**

**52.9** The Lieutenant Governor in Council may make regulations respecting the exercising of powers under section 52.6.

2002 c32 s12

**Termination of employment prohibited**

**52.91** No employer shall terminate, restrict or in any way discriminate against an employee for an absence from employment

- (a) that is by reason only of the employee providing a type of service under section 12.1,
- (a.1) that is in respect of and occurs during a public health emergency declared under section 52.1 and
  - (i) that is by reason only of the employee having being subject to a certificate issued pursuant to section 39, or
  - (ii) that is by reason only of the employee having been subject to an isolation order pursuant to section 44,
- (b) that is by reason only of the employee rendering aid under section 52.6(1)(b), or
- (c) that is authorized under section 52.6(1.1).

2002 c32 s12;2007 c23 s4;2020 c13 s12;2021 c15 s24

**52.92 to 52.992** Repealed RSA 2000 cP-37 s77.

**General****Confidentiality of communicable diseases information**

**53(1)** Information contained in any file, record, document or paper maintained by the Chief Medical Officer, a provincial health agency, a provincial health corporation, a health services delivery organization or an employee or agent acting on behalf of a provincial health agency, provincial health corporation or health services delivery organization that comes into existence through anything done under this Part and that indicates that a person is or was infected with a communicable disease shall be treated as private and confidential in respect of the person to whom the information relates and shall not be published, released or disclosed in any manner that would be detrimental to the personal interest, reputation or privacy of that person.

**(2)** For the purposes of assessing and improving the standards of care furnished to persons suffering from communicable diseases, compiling statistics with respect to communicable diseases, conducting research into communicable diseases, or for any reason

relating to communicable disease that the Chief Medical Officer considers to be in the interest of protecting the public health, the Chief Medical Officer may require any health practitioner to furnish the Chief Medical Officer with the following information:

- (a) a report containing the name and address of any patient of that health practitioner who is, was or may have been suffering from a communicable disease and a description of the diagnostic and treatment services provided to the patient;
- (b) medical or other records, or extracts or copies of them, in respect of that patient and in the possession of the health practitioner.

**(3)** Information obtained by the Chief Medical Officer, a provincial health agency, a provincial health corporation, a health services delivery organization or an employee or agent acting on behalf of a provincial health agency, provincial health corporation or health services delivery organization pursuant to this section shall be treated as private and confidential and, subject to subsections (4) and (4.1), shall not be published, released or disclosed in any manner that would be detrimental to the personal interest, reputation or privacy of the patient.

**(4)** Information obtained by the Chief Medical Officer, a provincial health agency, a provincial health corporation, a health services delivery organization or an employee or agent acting on behalf of a provincial health agency, provincial health corporation or health services delivery organization may be disclosed by the Chief Medical Officer, provincial health agency, provincial health corporation, health services delivery organization, employee or agent

- (a) to any person when required by law;
- (a.1) to any person where the Chief Medical Officer, provincial health agency, provincial health corporation, health services delivery organization, employee or agent believes on reasonable grounds that the disclosure will avert or minimize an imminent danger to the health or safety of any person;
- (b) to the person to whom the information relates or the person's legal representative;
- (c) in statistical form if the person to whom it relates is not revealed or made identifiable;

- (d) repealed RSA 2000 cH-5 s123;
- (e) to a person or body conducting an investigation or disciplinary proceedings pursuant to legislation governing a profession or occupation that is specified in the regulations when
  - (i) the information is requested by the person or body in accordance with the procedure governing the investigation or disciplinary proceedings, and
  - (ii) the person to whom the information relates consents to the disclosure.

**(4.1)** Information obtained by the Chief Medical Officer may be disclosed by the Chief Medical Officer to the Government of Canada, the government of another province or territory, the government of a foreign country or an agency of any of those governments for the purpose of addressing public health matters, patient safety, quality of care or the general public interest.

**(4.2) to (4.4)** Repealed RSA 2000 cP-37 s77.

**(5)** Subsection (1) does not prohibit the disclosure of information

- (a) to any person when required by law to do so,
- (a.1) to any person where the Chief Medical Officer, provincial health agency, provincial health corporation, health services delivery organization, employee or agent believes on reasonable grounds that the disclosure will avert or minimize an imminent danger to the health or safety of any person,
- (b) to any person with the written consent of the Minister, where in the Minister's opinion it is in the public interest that the information be disclosed to that person, or of the person to whom the information relates or the person's legal representative, or
- (c) to any person where the disclosure is necessary in the course of the administration of this Part.

RSA 2000 cP-37 ss53,77;RSA 2000 cH-5 s123;2002 c32 s12;2007 c23 s4;  
2009 c13 s4;2020 c13 s12;2021 c15 s26;2025 c10 s47(25)

**53.1 to 53.4** Repealed RSA 2000 cP-37 s77.

**Application to court****54(1)** Where a person

- (a) is prohibited by section 53 from publishing, releasing or disclosing information, or
- (b) refuses to disclose information that the person is permitted by section 53 to disclose,

the person to whom the information relates or the person's legal representative may apply for an order directing the person having the information to release it or a copy of it to the person to whom the information relates or the person's legal representative or to some other person named in the order.

**(2)** An application under subsection (1)

- (a) shall, if it is made in the course of any action or proceeding to which the person to whom the information relates or the person's legal representative is a party, be made on notice to a judge of the court in which the action or proceeding is taken, and
- (b) shall, in any other case, be made to a judge of the Court of King's Bench.

**(3)** Where the judge considers it appropriate to do so, the judge may order that the application under subsection (1) be heard in private.

**(4)** In an application under subsection (1), the onus of showing why the order should not be made for the release of the information is on the respondent to the motion.

RSA 2000 cP-37 s54;2009 c53 s149;AR 217/2022

**Offence**

**55** No person shall knowingly release, publish or disclose information contrary to section 53.

1984 cP-27.1 s65

**Provision of names**

**56(1)** A person suffering from a communicable disease referred to in section 20(2) shall, on request, provide the physician or sexually transmitted infections clinic responsible for the person's treatment with the names of all persons with whom the person has had sexual contact.

**(2)** Notwithstanding section 53, a physician who is provided with the names of contacts pursuant to subsection (1) shall immediately provide the information to a medical officer of health.

(3) Notwithstanding section 53, a medical officer of health may notify a person named as a contact pursuant to subsection (1).

RSA 2000 cP-37 s56;2016 c25 s22

#### **Delegation of authority**

**57** The Chief Medical Officer may in writing delegate to an employee of the Department any of the powers, duties and functions conferred or imposed on the Chief Medical Officer by this Act or the regulations.

1984 cP-27.1 s67;1998 c38 s19

**58** Repealed 2002 c32 s12.

### **Part 3.1 Public Health Plans and Health Impediments**

#### **Definitions**

**58.1** In this Part,

- (a) “health impediment” means a condition, thing or activity
  - (i) the cumulative effects of which, over time, are likely to adversely affect public health,
  - (ii) that causes chronic disease or disability in the population,
  - (iii) that interferes with or is inconsistent with the goals of public health initiatives respecting the prevention of injury or illness, including chronic disease or disability, in the population, or
  - (iv) that is associated with poor health within the population;
- (b) “population” includes a subclass within the population;
- (c) “public body” means
  - (i) the council of a municipality as defined in the *Municipal Government Act*;
  - (ii) a school jurisdiction as defined in the *Education Act*;
  - (iii) a provincial health agency;
  - (iv) a provincial health corporation;
  - (v) the board of a hospital other than a hospital that is owned and operated by a regional health authority;

- (vi) the operator of a continuing care home as defined in the *Continuing Care Act* other than a continuing care home that is owned and operated by a provincial health agency, provincial health corporation or health services delivery organization;
- (vii) the Health Quality Council of Alberta;
- (viii) any other public body prescribed in the regulations;
- (d) “public health plan” means a public health plan made under section 58.2;
- (e) “thing” includes
  - (i) tangible things, and
  - (ii) organisms, other than humans.  
2021 c15 s27;2022 cC-26.7 s77;2024 c10 s34;2024 c16 s20;  
2025 c10 s47(26)

**Public health plan**

**58.2(1)** The Minister may by order require a public body to make, in accordance with any regulations, a public health plan in respect of a specific issue or geographic area.

**(2)** The Minister may specify one or more of the following as the purpose or purposes of the public health plan:

- (a) to identify and address the health needs of particular groups within the population;
- (b) to monitor and assess the status of the health of the population, including through public health surveillance and monitoring indicators of, or factors influencing, the health of the population;
- (c) to identify, prevent and mitigate the adverse effects of diseases and disabilities, syndromes, psychosocial disorders, injuries and health impediments;
- (d) to achieve a purpose prescribed in the regulations.

**(3)** The Minister may require in an order under subsection (1) that the public body consult on the proposed public health plan with

- (a) each government specified in the order having jurisdiction in the geographic area to which the proposed public health plan applies, and

- (b) the provincial health agency established for each health services sector to which the proposed public health plan applies.

(4) The Minister may specify in an order under subsection (1) a date by which the public health plan must be completed and may by order extend that date whether or not the date previously specified has passed.

(5) A public body subject to an order under this section shall comply with the order.

2021 c15 s27;2025 c10 s47(27)

#### **Regulating health impediments**

**58.3(1)** This section applies to a person who is responsible for a condition or thing or who engages in an activity that is prescribed in the regulations for the purposes of this section as a condition, thing or activity that causes or is associated with a health impediment.

(2) A person referred to in subsection (1) shall

- (a) comply with any requirement or duty set out in the regulations respecting the condition, thing or activity,
- (b) not do anything that is prohibited by the regulations, and
- (c) ensure that employees are adequately trained and sufficiently equipped to comply with any requirement or duty set out in the regulations.

2021 c15 s27

## **Part 4 General**

### **Inspections and Orders**

#### **Inspection of place other than private dwelling**

**59(1)** An executive officer may inspect any public place for the purpose of determining the presence of a nuisance or determining whether this Act and the regulations are being complied with.

(2) An executive officer making an inspection under subsection (1) may

- (a) at any reasonable hour enter in or on the public place that is the subject of the inspection;
- (a.1) require any person to provide the name and contact information of any owner of the public place;



- (b) require the production of any books, records or other documents that are relevant to the purpose of the inspection and examine them, make copies of them or remove them temporarily for the purpose of making copies;
  - (c) make reasonable oral or written inquiries of any person who the executive officer believes on reasonable grounds may have information relevant to the subject-matter of the inspection;
  - (d) inspect and take samples of any substance, food, medication or equipment being used in or on the public place;
  - (e) perform tests, take photographs and make recordings in respect of the public place.
- (3) Where an executive officer removes any books, records or other documents under subsection (2)(b), the executive officer shall
- (a) give to the person from whom the items were taken a receipt for the items, and
  - (b) forthwith return the items to the person from whom they were taken when they have served the purpose for which they were taken.

RSA 2000 cP-37 s59;2021 c15 s28

**Inspection of private place**

**60** Where an executive officer believes on reasonable and probable grounds that a nuisance exists in or on a private place or that the private place or the owner of it is in contravention of this Act or the regulations, the executive officer may, with the consent of the owner or pursuant to an order under section 61,

- (a) enter in or on the private place at a reasonable hour and inspect it;
- (b) make reasonable oral or written inquiries of any person who the executive officer believes on reasonable grounds may have information relevant to the subject-matter of the inspection;
- (c) take samples of any substance, food, medication or equipment being used in or on the private place;
- (d) perform tests, take photographs and make recordings in respect of the private place.

1984 cP-27.1 s70;1998 c38 s17

**Assistance by police officer, expert**

**60.1** An executive officer making an inspection who enters in or on a public place or private place under section 59 or 60 may be accompanied by

- (a) a police officer whose presence is required by the executive officer for the purposes of assisting with the inspection, or
- (b) a qualified expert or professional whose presence is required by the executive officer for the purposes of inspecting and taking samples under section 59(2)(d) or 60(c) or performing tests, taking photographs or making recordings under section 59(2)(e) or 60(d).

2020 c5 s5

**Application to Court**

**61(1)** Where the owner of a public place or a private place refuses to allow an executive officer to exercise the executive officer's powers under section 59 or 60 or hinders or interferes with the executive officer in the exercise of those powers, the executive officer may apply to a judge of the Court of King's Bench for an order directing the owner to do or refrain from doing anything the judge considers necessary in order to enable the executive officer to exercise the executive officer's powers, and the judge may make the order accordingly.

**(2)** An application under subsection (1) may be made ex parte where the judge considers it proper to do so.

RSA 2000 cP-37 s61;2009 c53 s149;AR 217/2022

**Order**

**62(1)** An executive officer may issue a written order in accordance with this section if the executive officer has reasonable and probable grounds to believe, based on

- (a) an inspection of a public place under section 59 or a private place under section 60, or
- (b) a report or test, regardless of whether the report or test is required to be produced or performed under this Act, if a public place or private place was not inspected under section 59 or 60,

that a nuisance exists in or on the public place or private place, or that the place or owner of the place or any other person is in contravention of this Act or the regulations.

**(2)** An order shall be served on the person to whom it is directed and shall set out the reasons it was made, what the person is required to do and the time within which it must be done.

**(3)** Where the order is directed to a person who is not the registered owner, a copy of it shall also be served forthwith on the registered owner.

**(4)** An order may include, but is not limited to, provisions for the following:

- (a) requiring the vacating of the place or any part of it;
- (b) declaring the place or any part of it to be unfit for human habitation;
- (c) requiring the closure of the place or any part of it;
- (d) requiring the doing of work specified in the order in, on or about the place;
- (e) requiring the removal from the place or the vicinity of the place of anything that the order states causes a nuisance;
- (f) requiring the destruction of anything specified in the order;
- (g) prohibiting or regulating the selling, offering for sale, supplying, distributing, displaying, manufacturing, preparing, preserving, processing, packaging, serving, storing, transporting or handling of any food or thing in, on, to or from the place.

**(5)** Where the delay necessary to put an order under this section in writing will or is likely to increase substantially the hazard to the health of any person, the executive officer may issue the order orally.

**(6)** As soon as is reasonably possible after issuing an oral order under subsection (5), the executive officer shall serve a written version of the order in accordance with subsections (2) and (3).

**(7)** Where an order is issued under subsection (4)(a), (b) or (c), the executive officer shall ensure that a copy of the order, or in the case of an oral order, a notice of the requirements of the order, is posted in a conspicuous place at, on or near the public place or private place to which the order relates.

**(8)** A regional health authority shall maintain a record of all orders issued under subsection (4)(a), (b) or (c) and shall make the record

available for inspection by the public during the business hours of the main office of the regional health authority.

**(9)** If, in the course of an inspection under this Act, the executive officer is of the opinion that a condition of emergency exists due to the existence of a nuisance, the executive officer may, notwithstanding anything in this Act, forthwith take any steps the executive officer considers appropriate to remove or lessen the nuisance.

RSA 2000 cP-37 s62;2021 c15 s29

#### **Enforcement of order**

**62.1(1)** If a person to whom an order is directed under section 62 fails to carry out the order within the time limited by it in the case of an order that is not appealed, or within the time limited by the Board in the case of an order that is appealed to the Board, the executive officer or a person appointed by the executive officer may, together with any persons that are necessary, enter the public place or private place and carry out the order.

**(2)** A police officer may accompany an executive officer or a person appointed by the executive officer who is carrying out an order under subsection (1) for the purposes of assisting the executive officer or person appointed by the executive officer in carrying out the order.

2020 c5 s6

#### **Recovery of expenses**

**63(1)** In this section,

- (a) “clerk” means
  - (i) the chief administrative officer in the case of a city, town, village, summer village or municipal district,
  - (ii) the settlement administrator, in the case of a Metis settlement,
  - (iii) the Deputy Minister of the Minister responsible for the *Municipal Government Act*, in the case of an improvement district, or
  - (iv) the Deputy Minister of the Minister responsible for the *Special Areas Act*, in the case of a special area;
- (b) “expenses” includes legal fees and expenses.

**(2)** Repealed 2020 c5 s7.

(3) The expenses incurred by a regional health authority in carrying out an order under section 62.1 constitute a debt owing to the regional health authority from the person to whom the order is directed.

(4) Where a regional health authority carries out an order under section 62.1 and the person to whom the order is directed fails, within 60 days after a demand for payment, to pay the expenses incurred by the regional health authority, the secretary of the regional health authority may transmit to the clerk of the municipality in which the land concerned is located a statement setting out

- (a) the amount of the expenses,
- (b) the name of the registered owner of the land to which the order relates, and
- (c) the location of the land to which the order relates.

(5) On receipt of a statement under subsection (4), the municipality shall place the amount of the expenses incurred in carrying out the order on the tax roll as an additional tax against the land concerned and that amount

- (a) forms a lien on the land in favour of the municipality, and
- (b) is, for all purposes, deemed to be taxes imposed and assessed on land and in arrears under the *Municipal Government Act* from the date the amount was placed on the tax roll, and that Act applies to the enforcement, collection and recovery of the amount.

(6) Subsection (5) does not apply to a Metis settlement.

(7) Any amount collected by the municipality by virtue of subsection (5) shall be paid to the regional health authority.

(8) Where an amount recovered under this section by a regional health authority from a person other than the registered owner of the land to which the order relates is, as between that person and the registered owner, the responsibility of the registered owner, that person is entitled to recover the amount from the registered owner or to deduct the amount from any other amount due from that person to the registered owner.

(9) Where an amount recovered under this section by a regional health authority from the registered owner of land is, as between the registered owner and another person, the responsibility of that other person, the registered owner is entitled to recover the amount

from that other person or to deduct the amount from any other amount due from the registered owner to that other person.

RSA 2000 cP-37 s63;2020 c5 s7;2021 c15 s30

#### **Notice of health hazard**

**64(1)** When an order is issued under section 62, the regional health authority may cause to be filed with the Registrar of Land Titles a notice of health hazard against the registration of any person as transferee or owner of, or of any instrument affecting, the land that is the subject of the order, unless the instrument or certificate of title is expressed to be subject to that notice.

**(2)** A notice of health hazard registered under this section does not lapse and shall not be cancelled or withdrawn except on the receipt by the Registrar of a notice in writing from the regional health authority requesting cancellation or withdrawal.

**(3)** On registering a notice of health hazard, the Registrar shall notify the person against whose title the notice is registered and notify caveators and mortgagees when the addresses of those persons may be ascertained from the certificate of title.

1984 cP-27.1 s74;1996 c31 s24

#### **Notice of health hazard - Metis patented land**

**65(1)** When an order is issued under section 62 in respect of patented land as defined in the *Metis Settlements Act*, the regional health authority may submit a notice of health hazard to the Registrar of the Metis Settlements Land Registry and the Registrar shall record the notice against the Metis title register for the land that is subject to the order.

**(2)** A notice of health hazard recorded under this section does not lapse and shall not be cancelled except on the receipt by the Registrar of the Metis Settlements Land Registry, of a notice in writing from the regional health authority requesting cancellation.

**(3)** On recording a notice of health hazard, the Registrar of the Metis Settlements Land Registry shall notify the person against whose Metis title the notice is recorded and every person who has recorded an interest against the Metis title.

1998 c22 s32

### **Regulations**

#### **Regulations**

**66(1)** The Lieutenant Governor in Council may make regulations

- (a) prescribing communicable diseases for the purposes of this Act;

- (b) designating prescribed communicable diseases of which immediate notification is required for the purposes of sections 22(1)(a), 23(a)(i) and 27(1)(b);
- (c) respecting the prevention and control of the employment of persons who are carriers of or are infected with prescribed communicable diseases;
- (d) respecting isolation, quarantine, disinfection, disinfestation, decontamination, destruction of property, exclusion from employment, school or a public place and other special measures for the purposes of section 29(2);
- (d.1) repealed RSA 2000 cP-37 s77;
- (e) respecting the provision of samples to the Provincial Laboratory of Public Health for the purposes of section 24;
- (f) respecting the keeping of records for the purposes of Part 3, including, without limitation, regulations setting out
  - (i) who must keep records,
  - (ii) what information must be kept in the records and the form in which records must be kept, and
  - (iii) confidentiality provisions in respect of the records;
- (g) repealed 2021 c15 s31;
- (h) respecting the prevention, investigation and suppression among animals of infectious diseases communicable to humans;
- (i) respecting the quantity and manner of application of an agent to the eyes of newborn children to prevent the occurrence of prescribed communicable diseases;
- (j) respecting the licensing of embalmers and prescribing qualifications for admission of embalmers to practise in Alberta;
- (j.1) respecting the preparation, interment, disinterment and transportation of human corpses;
- (k) respecting qualifications of members of the Public Health Appeal Board, the Deputy Chief Medical Officer, medical officers of health, executive officers or community health nurses, including regulations providing differently for

different categories of medical officers of health or executive officers;

- (k.1) respecting additional qualifications of the Chief Medical Officer for the purposes of section 13(1.1)(d);
- (l) respecting the powers, duties and authority of executive officers or classes of executive officers;
- (m) respecting the kinds and basic standards of health promotional, preventive, diagnostic, treatment, rehabilitative and palliative services, supplies, equipment and care that must be provided by provincial health agencies or provincial health corporations and the conditions under which they are to be provided;
- (m.1) authorizing the Minister, a provincial health agency or a provincial health corporation to charge fees for goods and services provided by or on behalf of the Minister, provincial health agency or provincial health corporation in respect of carrying out duties and exercising powers under this Act, and respecting the amounts of the fees that may be charged;
- (n) respecting the services, supplies, equipment, care, drugs, medicines and biological agents that may be provided by the Minister for the purpose of section 12 and respecting the persons to whom and the conditions under which they may be provided;
- (o) establishing a mechanism to deal with appeals from decisions made in connection with the provision of services, supplies, equipment or care under regulations made under clause (n), and setting out what decisions may be appealed and the procedure to apply in an appeal;
- (o.1) respecting qualifications and requirements, types of services and individuals or classes of individuals who may provide services for the purposes of section 12.1, including regulations
  - (i) excluding individuals or classes of individuals from providing services under section 12.1, and
  - (ii) exempting individuals or classes of individuals from having the qualifications or meeting the requirements needed to provide services under section 12.1;
- (p) specifying professions and occupations for the purposes of section 53(4)(e);



- (q) repealed 2021 c15 s31;
- (r) respecting all aspects of the granting, cancellation and suspension of licences, permits or other approvals for any activity subject to regulations under this subsection;
- (s) specifying additional decisions for the purposes of section 5(1)(b) that may be appealed to the Board under that section;
- (t) prescribing information for the purposes of section 18.1(2.1)(c) or 18.2(2)(c);
- (u) prescribing forms for the purposes of this Act and the regulations;
- (v) authorizing the waiving or mitigation of the application of any of the provisions of the regulations in particular cases, respecting the circumstances under which the waiver or mitigation may be granted and respecting the conditions to which a grant of a waiver or mitigation is subject;
- (v.1) respecting reporting by health practitioners in respect of immunization;
- (v.2) respecting reporting by health practitioners in respect of adverse events following immunization;
- (v.3) respecting the handling, storage and transportation of biological agents intended for use in immunization, including, without limitation, regulations respecting actions to be taken after a contravention occurs in respect of the handling, storage and transportation of the biological agents;
- (w) prescribing public bodies for the purposes of section 58.1(c)(viii);
- (x) respecting public health plans, including regulations prescribing the purpose of a public health plan for the purposes of section 58.2(2)(d);
- (y) respecting health impediments and the regulation of health impediments, including regulations
  - (i) prescribing conditions, things or activities for the purposes of section 58.3(1),
  - (ii) respecting requirements or duties for the purposes of section 58.3(2), and

- (iii) prohibiting the doing of anything for the purposes of section 58.3(2)(b);
- (z) respecting the immunization of children attending or wishing to attend a child care program that is licensed under the *Early Learning and Child Care Act*;
- (aa) respecting the construction, location, operation, maintenance, equipping, sanitation and cleansing of food establishments engaged in selling, offering for sale, producing, supplying, distributing, displaying, manufacturing, preparing, preserving, processing, packaging, labelling, serving, storing, transporting or handling any food;
- (bb) respecting the establishment of standards for food, including standards for water and handling food, and providing for the destruction of any food that does not meet those standards;
- (cc) respecting the location, operation, maintenance, equipping, cleansing, disinfection and disinfestation of
  - (i) camps and campgrounds, and
  - (ii) water facilities, including wells, water fountains, cisterns, dugouts and water tanks;
- (dd) respecting the construction, location, operation, maintenance, disinfection, disinfestation and disposition of outdoor lavatories;
- (ee) respecting the handling and disposal of biomedical waste;
- (ff) respecting the location, operation, maintenance, equipping, cleansing, disinfection and disinfestation of public places;
- (gg) respecting the cleansing, disinfection and disinfestation of private dwellings;
- (hh) respecting the construction, inspection, operation, maintenance, equipping, cleansing, disinfection and disinfestation of public swimming pools;
- (ii) respecting the prevention and removal of nuisances;
- (jj) prescribing the maximum levels of contaminants permissible in air, water or soil;
- (kk) defining words or expressions used but not defined in this Act;

- (ll) respecting any other matter that the Lieutenant Governor in Council considers necessary to carry out the intent of this Act.

(2) Repealed 2021 c15 s31.

(3) A regulation made under this section may incorporate, adopt or declare in force a code, standard, guideline, schedule or body of rules, including a code, standard, guideline, schedule or body of rules developed by the Minister, relating to any matter in respect of which a regulation may be made under this section.

(4) Where a code, standard, guideline, schedule or body of rules is incorporated, adopted or declared in force by a regulation made under this section, the Minister shall ensure that a copy of the code, standard, guideline, schedule or body of rules is readily available to the public.

(4.1) The *Regulations Act* does not apply to a code, standard, guideline, schedule or body of rules incorporated, adopted or declared in force by a regulation made under this section.

(5) A code, standard, guideline, schedule or body of rules may be incorporated, adopted or declared in force by a regulation made under this section

- (a) in whole or in part or with modifications, and
- (b) as it reads on a specific day or as amended from time to time.

(6) Regulations under this section may be made applicable to a particular portion of Alberta only.

RSA 2000 cP-37 ss66,77;2002 c20 s6;2005 c13 s8;2007 cC-10.5 s29;  
2007 c18 s6;2007 c23 s4;2009 c13 s5;2016 c25 s23;2017 c22 s44;  
2020 c13 s12;2020 c31 s40;2021 c15 s31;2025 c10 s47(31)

## Miscellaneous

### Protection from liability

**66.1(1)** No action for damages may be commenced against

- (a) the Crown or a Minister of the Crown,
- (b) a regional health authority or a member, employee or agent of a regional health authority,
- (c) an employee under the administration of the Minister,
- (c.1) a provincial health agency or a member, employee or agent of a provincial health agency,

- (d) the Chief Medical Officer, the Deputy Chief Medical Officer, an executive officer or a medical officer of health,
- (e) a health practitioner,
- (f) a teacher, a person in charge of an institution or a medical director of a facility, or
- (g) repealed 2008 cH-5.3 s24,
- (h) a provincial health corporation or a member, employee or agent of a provincial health corporation,
- (i) a health services delivery organization or a member, employee or agent of a health services delivery organization

for anything done or not done by that person in good faith while carrying out duties or exercising powers under this or any other enactment.

**(2)** No action for damages may be commenced against any person or organization acting under the direction of the Crown, a Minister of the Crown, the Chief Medical Officer, the Deputy Chief Medical Officer or a medical officer of health for anything done or not done by that person or organization in good faith directly or indirectly related to a public health emergency while carrying out duties or exercising powers under this or any other enactment.

2002 c32 s12;2007 c23 s4;2008 cH-4.3 s24;2024 c10 s34;  
2024 c16 s20;2025 c10 s47(32)

#### **Court enforcement**

**66.2(1)** The Court of King's Bench may, on application by the Minister, a regional health authority or, in the case of section 59, 60 or 61, an executive officer, make any order it considers necessary to enforce this Act.

**(2)** An application under this section may be heard in any manner the Court considers appropriate and may be heard before the application is filed.

**(3)** An interim order may be made under this section on an ex parte application.

2007 c23 s4;2009 c53 s149;AR 217/2022

#### **Crown's right of recovery**

**67** The Crown in right of Alberta is entitled to recover the Crown's cost of public health services under the *Crown's Right of Recovery Act* or the *Opioid Damages and Health Care Costs Recovery Act*.

RSA 2000 cP-37 s67;2009 cC-35 s59;2019 cO-8.5 s18

**Service of documents**

**68(1)** Where this Act or the regulations require or permit the service of an order, notice or other document on a person, then unless this Act otherwise provides, the order, notice or other document is sufficiently served if it is

- (a) served personally on the person,
- (b) sent by registered mail to the person at the person's last known address,
- (b.1) served by electronic means, including facsimile, on a person at the electronic address provided by that person, where the electronic means provides
  - (i) the sender with confirmation that transmission to the electronic address of the recipient of the order, notice or other document was successfully completed, and
  - (ii) the recipient with the order, notice or other document in a form that is usable for subsequent reference,

or

- (c) published in a newspaper in accordance with subsection (2).

**(2)** Where the person serving an order, notice or other document, after having taken reasonable steps for the purpose, is unable to locate the person to be served or to determine that person's actual address, the order, notice or other document may be served by publishing it in at least 2 issues, at least a week apart, of a newspaper having general circulation

- (a) in the place where the person to be served had that person's last known address according to the records or other information available to the person serving the order, notice or other document, and
- (b) if the notice, order or other document relates to the property of the person to be served or is given in proceedings that relate to that person's property, in the place where the property is located.

RSA 2000 cP-37 s68;2020 c5 s8

**Validity of documents**

**69** A certificate, notice, order, warrant or other form issued under this Act or the regulations shall not be held to be insufficient or invalid by reason only of any irregularity, informality or

insufficiency in it or in any proceedings in connection with its issuance.

1984 cP-27.1 s77

**70** Repealed 2021 c15 s32.

#### **Obstruction**

**71** No person shall obstruct, molest, hinder or interfere with a person in the execution of any duty imposed or in the exercise of any power conferred on the person by this Act or the regulations.

1984 cP-27.1 s79

#### **Destruction of notice**

**72** No person shall conceal, deface, destroy or remove any notice posted for public information under this Act or the regulations.

1984 cP-27.1 s80

#### **Penalty**

**73(1)** A person who contravenes this Act, the regulations, an order under section 62 or an order of a medical officer of health or physician under Part 3 is guilty of an offence.

**(2)** A person who contravenes an order under section 62 or an order of a physician under Part 3 is liable to a fine of not less than \$100 and not more than \$5000 for each day or part of a day during which the contravention occurs or continues.

**(3)** A person who contravenes this Act, the regulations or an order of a medical officer of health under Part 3 is, if no penalty in respect of that offence is prescribed elsewhere in this Act, liable to a fine of not more than \$100 000 in the case of a first offence and \$500 000 in the case of a subsequent offence.

**(4)** Where a person is convicted of an offence under this Act, the judge, in addition to any other penalty the judge may impose, may order the person to comply with the provision of this Act or the regulations or the order for the contravention of which the person was convicted.

RSA 2000 cP-37 s73;2020 c5 s9

#### **Prosecution time limit**

**73.1(1)** A prosecution of an offence under this Act or the regulations may not be commenced more than 3 years after the day on which evidence of the offence first came to the attention of an executive officer.

**(2)** This section applies only in respect of offences that are committed on or after the day on which this section comes into force.

2021 c15 s33

**Publishing reports and order**

**74** A regional health authority or the Minister may publish the following documents in the manner the regional health authority or the Minister, as the case may be, considers appropriate:

- (a) a report arising from an inspection under this Part;
- (b) an order issued under section 62.

RSA 2000 cP-37 s74;2009 c13 s6

**Lieutenant Governor in Council authority**

**74.1** Subject to section 29(2.12) and (2.13), and notwithstanding any other provision in this Act, the Lieutenant Governor in Council may by order reverse or vary any decision of any decision-maker made under this Act.

2023 c11 s3

**Paramountcy**

**75** Except for the *Alberta Bill of Rights*, this Act prevails over any enactment that it conflicts or is inconsistent with, including the *Health Information Act*, and a regulation under this Act prevails over any other bylaw, rule, order or regulation with which it conflicts.

RSA 2000 cP-37 s75;RSA 2000 cH-5 s123

**76** Repealed RSA 2000 cP-37 s77.

**Review of Act**

**76.1(1)** At least once every 10 years, the Minister shall commence a review of this Act.

**(2)** The first review must commence within 10 years after the date on which this section comes into force.

2021 c15 s34

**Repeals**

**77** The following provisions are repealed on December 31, 2021:

section 1(1)(e.1) and (2);  
section 33(2.1) and (2.2);  
section 52.6(1.1)(c) and (1.2);  
section 52.61;  
sections 52.92 to 52.992;  
section 53(4.2) to (4.4);  
sections 53.1 to 53.4;  
section 66(1)(d.1);  
section 76.

2020 c13 s12;2021 c15 s35



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